

[Home](#) | [JAMA](#) | [New Online](#)**Editorial** | AI in Medicine**FREE**

Updated Guidance for Author Use of AI in Medical Publication

Annette Flanagan, RN, MA¹; Roy H. Perlis, MD, MSc^{2,3}; Kirsten Bibbins-Domingo, PhD, MD, MAS¹[» Author Affiliations](#) | [Article Information](#)[Cite](#) [Permissions](#) [Metrics](#)**JAMA****Published Online: August 10, 2026**

doi: 10.1001/jama.2026.16613

[RELATED ARTICLES](#) [FIGURES](#)

Artificial intelligence (AI) is reshaping research and scientific publishing at an unprecedented pace, with extraordinary promise. We as editors embrace AI as a tool that can be helpful to researchers and authors by reducing the burdens of some tasks and improving efficiency in some contexts. However, as use of AI in scientific publishing grows exponentially,¹⁻³ so do concerns about such use.³⁻¹⁰ Journals have adopted policies regarding appropriate and responsible use of AI by authors, reviewers, and editors, which range from encouragement of AI use, provided that such use is disclosed, to restrictions on some specific types of use.¹¹⁻¹³ Yet very low rates of disclosure of AI use by authors have been reported.¹⁴⁻¹⁷ Reasons reported by authors for failure to disclose use of AI include concern that it may lead to biased editorial assessments and unfavorable editorial decisions and lack of understanding of journal policies of what should be disclosed.¹⁸ Recognizing that transparency is central to the integrity of the scholarly record, recommendations for more uniform disclosure of AI use by authors may be helpful. Efforts to develop

[Sections](#)[PDF](#)[Share](#)

interim, we offer guidance to authors for responsible use, highlighting what is permitted and not permitted and what needs to be disclosed ([Table](#) and [Box](#)).

Advertisement

View Large

Download

Table. JAMA Network Journals Guidance for Acceptable Use of AI

 JAMA Network Journals Guidance for Acceptable Use of AI

Box. Use of AI in Manuscript and Content Preparation and Submission

Responsibility: Authors are responsible for the accuracy of all content submitted to the journal.

Authors must review and confirm the accuracy of all content generated by artificial intelligence (AI), large language models (LLMs), chatbots, or similar technologies and must be accountable and take full responsibility for all content.

Disclosure: Use of AI, LLMs, chatbots, or similar technologies to assist with content creation, revision, and formatting must be reported in the Acknowledgment section.

References: AI, LLM, and chatbots should not be used to generate or format references because these tools may cause problems with the accuracy and integrity of cited references; standard reference managers may be used instead. Check all references for completeness, accuracy, and integrity.

Opinion and Correspondence Manuscripts: Use of AI, LLM, and chatbots to draft Opinion manuscripts, Letters to the Editor, and online Comments is not permitted.

Clinical Images, Illustrations, and Multimedia: Submission of clinical images and clinical illustrations created or manipulated by AI, LLM, and chatbots is not permitted unless part of formal research design or methods, which must be fully disclosed. This policy also applies to the submission of clinical video and audio files.

Failure to Disclose: If incomplete disclosure or failure to disclose use of AI is detected during the editorial process or after publication, content may be rejected or subject to postpublication

If AI, LLM, or chatbots were used to assist with content creation, revision, or formatting, the following should be reported in the Acknowledgment section:

- Name of the AI software platform, program, or tool
- Version and extension numbers
- Manufacturer
- Date(s) of use
- A brief description of how the AI was used and on what portions of the manuscript or content
- Confirmation that the author(s) takes responsibility for the accuracy and integrity of the content generated

Note this guidance does not apply to basic tools for checking grammar or spelling.

In some cases, disclosure policies may not be sufficient to address concerns about AI-generated content that is incorrect or even fabricated (ie, hallucinated). Such policies also cannot stem the tide of AI-generated manuscript submissions with little to no scholarly or clinical value. For example, across the JAMA Network we have received manuscripts with multiple inaccurate or nonexistent references, a common problem when authors use AI to identify or format their references. Even frontier models can introduce realistic-looking references, reflecting real authors and titles that support a particular point but that do not exist. As a result, we have updated our guidance to advise authors to not use AI, including large language models (LLMs), to generate, format, or otherwise manage references. We encourage the use of standard reference managers instead, and we remind authors to check all references for completeness, accuracy, and integrity. The references are an important section of any manuscript, and authors are as responsible for this section as for any other.

Our research and experience also demonstrate that author use of AI to assist with manuscript writing is often associated with specific types of articles, such as Opinion articles and Letters to the Editor.¹⁴ We have seen marked increases in submissions of these types of articles, occasionally from the same authors, who may not have expertise in the topics they are addressing. These submissions are often formulaic and lack useful insights.²³ Thus, we have revised our guidance to authors to inform them that use of AI, including LLMs, to draft Opinion manuscripts, Letters to the Editor, and online Comments is not permitted. Use of AI tools and software to check grammar and spelling or for translation is permitted, but only after the initial text is written by humans. Authors must understand the risks of putting their content into an AI model and take full responsibility for any content changed by such tools.

[Sections](#)[PDF](#)[Share](#)

We also share concerns about the accuracy, clinical fidelity, and demographic representational bias in AI-generated images.²⁴ Therefore, we have updated our guidance regarding use of AI for clinical images and illustrations to more explicitly state that submission of clinical images and clinical illustrations created or manipulated by AI is not permitted unless part of formal research design or methods, which must be fully disclosed. Preparation of scientific images (clinical images, radiographic images, micrographs, gels, etc) for publication must preserve the integrity of the image data. Digital adjustments of brightness, contrast, or color applied uniformly to an entire image are permissible as long as these adjustments do not selectively highlight, misrepresent, obscure, or eliminate specific clinical or scientific elements in the original image. Replacement of nonclinically relevant background with a solid color in photographs is permitted. Any such adjustment or background replacement must be fully described in the figure legend along with indication of the AI or software (name and version number) used, and the original unedited photographs must be provided for editorial review. Use of AI to generate graphic visualization of research data or results is permitted, provided that the AI or software (name and version number) used is included in the figure legend. The submission of clinical video or audio created or manipulated by AI is not permitted unless part of formal research design or methods, which must be fully disclosed.

Beyond these disclosure requirements and restrictions on AI use for specific types of content, our guidance reaffirms the foundational tenet that authors must be responsible for the accuracy of all content submitted to journals, whether AI generated or assisted or produced solely by humans. We recognize the rapidly evolving and extraordinary influence of AI on research and scientific publishing, with concomitant efficiencies, as well as threats and concerns. When AI is used as part of a scientific study, we ask authors to follow our previously released guidance for complete and transparent reporting of the methods and results to address potential biases, limitations, generalizability, and reproducibility.²² Our updated guidance for appropriate use of AI in manuscript and other content preparation, and for what is not permitted, is intended to promote and preserve accuracy and integrity of publication at a time when upholding the trustworthiness of the scientific record is more critical than ever. We have included this updated guidance in the *JAMA* and JAMA Network journals' Instructions for Authors,²⁵ and we will monitor and continue to update this guidance to best support authors and the quality and transparency of publication.

Article Information

Corresponding Author: Annette Flanagin, RN, MA, *JAMA* and JAMA Network (annette.flanagin@jamanetwork.org).

Published Online: August 10, 2026. doi:10.1001/jama.2026.16613



Sections



PDF



Share

References

1. Kusumegi K, Yang X, Ginsparg P, de Vaan M, Stuart T, Yin Y. Scientific production in the era of large language models. *Science*. 2025;390(6779):1240-1243. doi:[10.1126/science.adw3000](#)
[PubMed](#) | [Google Scholar](#) | [Crossref](#)
2. Naddaf M. How much of the scientific literature is generated by AI? *Nature*. Published online May 5, 2026. doi:[10.1038/d41586-025-03504-8](#)
[PubMed](#) | [Google Scholar](#) | [Crossref](#)
3. Society for Scholarly Publishing. Pulse Check report: AI in scholarly publishing. Published January 1, 2026. Accessed August 4, 2026. <https://customer.sspnet.org/SSP/ssp/resources/Pulse-Poll-January-2026.aspx>
4. Resnik DB, Hosseini M. Hallucinated citations produced by generative artificial intelligence may constitute research misconduct when citations function as data in scholarly papers. *Account Res*. Published online March 15, 2026. doi:[10.1080/08989621.2026.2645390](#)
[Google Scholar](#) | [Crossref](#)
5. Ng JY, Bhavsar D, Dhanvanthry N, et al. Attitudes and perceptions towards the use of artificial intelligence chatbots in medical journal peer review: a protocol for a large-scale, international cross-sectional survey. *Eur Sci Ed*. 2025;51:e159921. doi:[10.3897/ese.2025.e159921](#)
[Google Scholar](#) | [Crossref](#)
6. Carobene A, Padoan A, Cabitza F, Banfi G, Plebani M. Rising adoption of artificial intelligence in scientific publishing: evaluating the role, risks, and ethical implications in paper drafting and review process. *Clin Chem Lab Med*. 2023;62(5):835-843. doi:[10.1515/cclm-2023-1136](#)
[PubMed](#) | [Google Scholar](#) | [Crossref](#)
7. Suchak T, Aliu AE, Harrison C, Zwiggelaar R, Geifman N, Spick M. Explosion of formulaic research articles, including inappropriate study designs and false discoveries, based on the NHANES US national health database. *PLoS Biol*. 2025;23(5):e3003152. doi:[10.1371/journal.pbio.3003152](#)
[PubMed](#) | [Google Scholar](#) | [Crossref](#)
8. Hosseini M, Horbach SPJM. Fighting reviewer fatigue or amplifying bias? considerations and recommendations for use of ChatGPT and other large language models in scholarly peer review. *Res Integr Peer Rev*. 2023;8(1):4. doi:[10.1186/s41073-023-00133-5](#)
[PubMed](#) | [Google Scholar](#) | [Crossref](#)
9. Lin Z. Hidden prompts in manuscripts exploit AI-assisted peer review. *arXiv*. Preprint posted online July 8, 2025. doi:[10.48550/arXiv.2507.06185](#)

[Sections](#)[PDF](#)[Share](#)

10. Topaz M, Roguin N, Gupta P, Zhang Z, Peltonen LM. Fabricated citations: an audit across 2.5 million biomedical papers. *Lancet*. 2026;407(10541):1779-1781. doi:[10.1016/S0140-6736\(26\)00603-3](#)
[PubMed](#) | [Google Scholar](#) | [Crossref](#)
11. Flanagin A, Kendall-Taylor J, Bibbins-Domingo K. Guidance for authors, peer reviewers, and editors on use of AI, language models, and chatbots. *JAMA*. 2023;330(8):702-703. doi:[10.1001/jama.2023.12500](#)
[PubMed](#) | [Google Scholar](#) | [Crossref](#)
12. Perlis RH, Christakis DA, Bressler NM, et al. Artificial intelligence in peer review. *JAMA*. 2025;334(17):1520-1522. doi:[10.1001/jama.2025.15827](#)
[PubMed](#) | [Google Scholar](#) | [Crossref](#)
13. International Committee of Medical Journal Editors. Recommendations for the conduct, reporting, editing, and publication of scholarly work in medical journals: use of artificial intelligence in publishing. Updated January 2026. Accessed June 25, 2026.
<https://www.icmje.org/recommendations/browse/artificial-intelligence/>
14. Perlis RH, Flanagin A, Kendall-Taylor J, Berkwits M, Bibbins-Domingo K. Author disclosure of use of AI in submissions to 13 JAMA Network journals. *JAMA*. 2026;335(8):717-719. doi:[10.1001/jama.2025.25300](#)
[PubMed](#) | [Google Scholar](#) | [Crossref](#)
15. AlFayyad I, Zeegers MP, Bouter L, Macdonald H, Schroter S. Self-disclosed use of AI in research submissions to *BMJ* journals. *JAMA*. 2026;335(8):719-721. doi:[10.1001/jama.2025.25688](#)
[PubMed](#) | [Google Scholar](#) | [Crossref](#)
16. Malani PN, Ross JS. AI use in research and the need for continued guidance. *JAMA*. 2026;335(8):673. doi:[10.1001/jama.2025.26845](#)
[PubMed](#) | [Google Scholar](#) | [Crossref](#)
17. Jonah Barrett D, Heng R, Perchik JD. Documenting disclosure: limited reporting of generative AI usage in radiology research manuscripts. *Acad Radiol*. 2025;32(10):5686-5692. doi:[10.1016/j.acra.2025.06.057](#)
[PubMed](#) | [Google Scholar](#) | [Crossref](#)
18. BaHammam AS. The transparency paradox: why researchers avoid disclosing AI assistance in scientific writing. *Nat Sci Sleep*. 2025;17:2569-2574. doi:[10.2147/NSS.S568375](#)
[PubMed](#) | [Google Scholar](#) | [Crossref](#)



Sections



PDF



Share

20. STM. Recommendations for a classification of AI use in academic manuscript preparation. Published September 2025. Accessed June 25, 2026. https://s3.eu-west-2.amazonaws.com/stm.offloadmedia/wp-content/uploads/2025/04/23020709/STM_AI_Classification_Recs_19_Sept2025-1.pdf
21. International Science Council. AI disclosure in research: towards a global reporting standard. Accessed June 25, 2026. <https://council.science/our-work/ai-disclosure-in-research/>
22. Flanagin A, Pirracchio R, Khera R, Berkwits M, Hsuen Y, Bibbins-Domingo K. Reporting use of AI in research and scholarly publication—JAMA Network guidance. *JAMA*. 2024;331(13):1096-1098. doi:10.1001/jama.2024.3471
[PubMed](#) | [Google Scholar](#) | [Crossref](#)
23. Rasmussen TE. Preserving relational letter writing in the era of artificial intelligence. *JAMA Surg*. Published online June 24, 2026. doi:10.1001/jamasurg.2026.2165
[PubMed](#) | [Google Scholar](#) | [Crossref](#)
24. Alon L, Shoval DH, Levkovich I. Bias, representation, and clinical fidelity in AI-generated images for medical education: a systematic literature review. *NPJ Digit Med*. 2026;9(1):469. doi:10.1038/s41746-026-02608-3
[PubMed](#) | [Google Scholar](#) | [Crossref](#)
25. JAMA Network. *JAMA*: instructions for authors. Updated July 29, 2026. Accessed August 4, 2026. <https://jamanetwork.com/journals/jama/pages/instructions-for-authors>

[View Full Text](#) | [Download PDF](#)



Sections



PDF



Share